



CP PHARMA

VIDIPxx-03

CP PHARMA DIPHENHYDRAMINE EXPECTORANT (SUGAR FREE)

DESCRIPTION

Brown liquid with raspberry flavour

COMPOSITION

Each 5 ml contains:
Diphenhydramine Hydrochloride 14.0 mg
Ammonium Chloride 135.0 mg
Sodium Citrate 57.0 mg

PHARMACODYNAMICS

Diphenhydramine Hydrochloride

Antihistaminic

Diphenhydramine which is used in the treatment of allergy act by competing with histamine for H1-receptor sites on effector cells. It thereby prevents, but do not reverse, responses mediated by histamine alone. The antimuscarinic actions of diphenhydramine provide a drying effect on the nasal mucosa.

Antitussive

Diphenhydramine suppresses the cough reflex by a direct effect on the cough center in the medulla of the brain.

Ammonium Chloride

Expectorant

The ammonium ion supposedly exerts an expectorant action. Its action is doubtful as to whether its irritant action on the gastric mucous membrane contributes to any expectorant action.

Sodium Citrate Dihydrate

Expectorant

Sodium citrate is used for its action in rendering mucous less viscous, hence a useful ingredient in expectorant.

PHARMACOKINETICS

- Absorption: Diphenhydramine and sodium citrate is readily absorbed after oral administration. When taken orally, ammonium chloride is rapidly absorbed from the gastrointestinal tract.
- Blood Concentration: After an oral dose of 50 mg of the hydrochloride as a solution, plasma concentrations of 50 to 54 ng/ml are attained at 1 hour; at 2 hours, the respective plasma concentrations are 64 to 70 ng/ml.
- Half Life: The plasma half life after an oral dose of diphenhydramine is 13 to 21 hours.
- Distribution: 98% of diphenhydramine is bound to plasma proteins.
- Metabolic Reactions: Diphenhydramine is extensively metabolised. About 50% of an oral dose is metabolised in the liver before reaching the general circulation. Reactions include N-dealkylation and deamination.
The ammonium ion (from ammonium chloride) is converted into urea in the liver; the anion thus liberated into the bloodstream and extracellular fluids causes a metabolic acidosis and decreases the pH of the urine.
Sodium citrate is oxidised in the tissues and is partly excreted as carbon dioxide.
- Excretion: In the urine up to 3% of a dose is excreted unchanged, up to 13% as basic amines and up to 65% as diphenylmethane metabolites. The major urinary metabolite appear to be diphenylmethoxyacetic acid in free or conjugated form.
Ammonium chloride, as its urea metabolite is excreted through the urine.

INDICATIONS

It is indicated as an expectorant for control of coughs due to colds or allergy.

CONTRAINDICATIONS

- Avoid in patients known to be hypersensitive to antihistamines.
- Not recommended in patients with hepatic or renal function impairment and in nursing mothers.
- It should not be used in newborn or premature infants because of the higher risk of adverse effects with antihistamines.

WARNINGS AND PRECAUTIONS

- Caution in patients with glaucoma, prostatic hypertrophy, cardiovascular disease, hypertension or acute asthma.
- This medication may cause drowsiness. Caution when driving or operating machinery. Avoid alcoholic beverages.
- Safety for use in pregnancy has not been established.
- Not to be used in children less than 2 years of age.
- To be used with caution and doctor's advice in children 2 to 6 years of age.
- Effects on ability to drive and use machines.
This product may cause drowsiness. If affected, the patient should not drive or operate machinery.

PREGNANCY AND LACTATION

Safety for use in pregnancy and lactation has not been established.

DRUG INTERACTIONS

- Concurrent use with alcohol, other CNS depressants, tricyclic antidepressants, or antimuscarinics may potentiate the effects of either these medications or diphenhydramine.
- Co-administration of monoamine oxidase (MAO) inhibitors may prolong and intensify the antimuscarinic effects of diphenhydramine and is generally not recommended.

MAIN SIDE/ADVERSE EFFECTS

- Drowsiness, nausea, vomiting.
- Geriatric patients are especially susceptible to the anticholinergic side effects, such as dryness of mouth and urinary retention (especially in males), of the antihistamines. If these side effects occur and continue or are severe, medication should probably be discontinued.

OVERDOSAGE AND TREATMENT

Clinical features: Drowsiness, headache, tachycardia, disorientation, staggering gait, hallucinations, respiratory depression, convulsions. Treat overdose by emesis or gastric lavage, if appropriate, with the necessary symptomatic and supportive measures, if required.

DOSAGE AND ADMINISTRATION

Unless otherwise prescribed by the physician:

Adults and children over 12 years:

5 to 10 ml every 4 or 6 hours (not to exceed 60 ml in 24 hours).

Children 6 to 12 years:

2.5 to 5 ml every 4 to 6 hours (not to exceed 30 ml in 24 hours).

Children 2 to 5 years:

2.5 ml every 4 to 6 hours (not to exceed 20 ml in 24 hours).

Take medication with a full glass of water. Do not exceed the stated dose.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage: Store below 30°C. Protect from light.

Discard any remaining contents 30 days after first opening the bottle.

For oral use.

Presentation/Packing: Bottle of 120 ml

Manufactured by: HOVID Bhd.

121, Jalan Tunku Abdul Rahman (Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Product Registration Holder: Hovid Nutriworld Sdn. Bhd.

121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.

To be sold in: Caring Pharmacy Retail Management Sdn. Bhd.

No. 1, Jalan 51/203A, Kawasan Perindustrian Tiong Nam, Seksyen
51, 46050 Petaling Jaya.

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